

BEYOND THE CLINIC WALLS

Engaging Communities in Healthcare

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“Health happens everywhere else.”

A field reflection on trust, co-design, storytelling and the social realities that shape health—drawn from lived experience in community and public health practice.

The Story of a Simple Goodbye

One ordinary day in 2006, at our office in Davangere, I noticed Devi sitting quietly with red, watery eyes. Devi was a Peer Educator and a street-based sex worker. When I gently asked why she had drunk so much the previous night, she wouldn't look at me at first. Then, she turned, her voice almost tears in the corner of her eyes: "Because you did not tell me goodbye before leaving."

In that moment, beneath the smell of alcohol and the redness in her eyes, I could see the depth of her hurt and the purity of her affection. I wondered why someone so bold, street-smart, fiercely independent, and always dignified in her appearance sought my acknowledgement so deeply.

The previous evening, I had accompanied her into the community to understand the sex worker network. She walked me through narrow lanes, introduced me to women in the area as part of our work. In the rush to leave, I had forgotten to say goodbye.

That pierced straight through me. Years later, I understand it differently: For people who have lived with vulnerability, stigma, exclusion, and invisibility, being acknowledged matters.

Devi's story was just the beginning. In my career in the development sector, I had many such personal experiences from the field where real human stories broke through clinical boundaries and completely rebuilt my perspective on what true community engagement actually means.

I am not going to analyse existing academic public health or community engagement theories here, but rather offer a reflection of hard-won lessons, missteps, and successes from my own experience in co-creating community health solutions.

The Trust Deficit in Empty Rooms

Community engagement is not built only through programs, strategies, or innovations. It is built in the quiet human moments that communicate respect, trust, and belonging. Sometimes, a simple goodbye can mean everything.

Imagine a local clinic launching a state-of-the-art, free diabetes screening program in an underserved neighborhood. The clinical team sits in a pristine room, waiting. The flyers are printed; the budget spent. Yet, hours pass, and the chairs remain empty.

To the healthcare force, it is "patient non-compliance." But to anyone living in that neighborhood, the reality is obvious: the clinic is a one-bus transfer away, the timings conflict with working hours, and the community has historically felt dismissed by institutional medicine.

The stethoscope around a doctor's neck can only hear what happens inside the exam room. But health? Health happens everywhere else.

This is the trust deficit in modern healthcare. We may have mastered the science of treating disease, but we are still learning the art of engaging communities. True healthcare transformation doesn't start with modern medical devices or a press release, but by shifting our mindset from treating patients to empowering communities.

Key Strategies for Meaningful Community Engagement

1. Rebuilding Trust Through "Micro-Communities"

During our work with NCD and CPHC programs in the bustling neighborhoods of Singasandra, Bengaluru, we quickly realized that a clinical banner alone wasn't enough to bring people through the door. We sought out the heartbeat of the community: a local Self-Help Group (SHG) leader and a dedicated School Development and Monitoring Committee (SDMC) member, who held the “invisible keys” of trust in the neighborhood.

By partnering with them, the entire dynamic of our health camps shifted completely. Instead of sterile, intimidating clinic setups, we gathered on the familiar turf of local community spaces, steered by the passionate energy of micro-groups—from formal networks of Mahila Arogya Samitis (MAS) to informal neighborhood SHGs and women's clubs. The community turned up in overwhelming numbers, proving that health initiatives only truly come alive when anchored in spaces and voices people already call home.

- **Find the Cultural Bridges:** Meet communities where they already gather—whether that is a local neighborhood center, a public space, Anganwadi Centre, or a community member's terrace.
- **The Trust Transfer:** Partnering with trusted local figures and grassroots leaders gives healthcare messages instant authenticity that money cannot buy.

2. Moving from “Informing” to “Co-Designed”

Communities do not truly participate in initiatives that are designed without them. In the Arogyasangama project by RDPR for empowering Panchayati Raj Institutions (PRIs), KHPT co-designed workshops directly with PRI members and local village communities. As a result, the project saw immense participation, even from passive women members who previously waited for male agreement on decisions.

Similarly, before supporting the PHCs of Bengaluru and Mysuru to reactivate Mahila Arogya Samitis under NHM, consultation workshops were conducted with existing MAS members, ASHAs, community women, and stakeholders. Instead of arriving with fixed plans, listening to their realities led to co-designing the approach together.

- **The Blueprint Rule:** If a community isn't involved in designing a health initiative, they won't participate in it.
- **Flattening the Hierarchy:** Bring community members into planning early. Run digital polls, host raw listening sessions, and ask: “What are we misunderstanding about your daily life?”

3. The Power of Storytelling and Participatory Approaches

During field training experiences, relying on technical language or classroom teaching often fails. Designing training modules in participatory and interactive game-based capsule formats allows content to be accessible to everyone—literate or illiterate, migrants facing language barriers, and diverse age groups.

Using stories, visuals, role plays, games, and interactive conversations creates safe and inclusive spaces where people feel confident to ask questions, share experiences, and learn from one another. Co-created community engagement training modules ground health principles in interactive learning, strengthening community ownership and improving access to services.

While digital platforms like WhatsApp, Facebook, and video services serve as new town squares, systems like automated IVRFS are often met with passive responses if they lack human connection. A town square is meant for dialogue, not lecturing.

- **Democratize Knowledge:** Translate dense clinical jargon into empathetic, everyday language. If health literacy isn't accessible, it isn't useful.

- **Humanize the Professional:** People connect with human faces and shared experiences. Sharing the “why” behind a clinical journey dismantles the intimidating wall of medicine.

4. Holistic Engagement: Addressing Health Beyond the Clinic

While interacting with a support group of people living with Non-Communicable Diseases (NCDs), an elderly man was asked why he continued eating rice every day despite knowing it was not ideal for his diabetes. He smiled helplessly and said:

“Who will prepare a separate diet for me? That’s what my daughter-in-law can prepare. She leaves early for work, takes care of the children, and manages the house.”

This response highlights a critical reality: what appears as “non-compliance” is often shaped by caregiving burdens, livelihoods, family dynamics, time, and access. Health decisions are rarely individual choices alone—they are deeply connected to social and economic circumstances.

We cannot effectively talk to a community about preventative wellness if they are actively facing housing insecurity, violence, hunger, or lack of reliable transportation. These Social Determinants of Health (SDOH) dictate a community's capacity to care for themselves.

Healthcare professionals must look outside clinical boundaries and partner with local businesses, non-profits, and grassroots advocates to address root systemic issues including poverty, stigma, unsafe environments, migration, gender inequities, and structural access.

From Broadcast to Participation

Traditional Approach	Co-Designed Approach
Focuses on top-down instructions	Focuses on active listening and mutual creation
Treats communities as passive beneficiaries	Empowers communities as active co-creators
Higher risk of non-compliance and mistrust	Builds sustainable ownership and shared leadership

Conclusion: A Legacy of Connection

Community engagement is not a checkbox in an intervention strategy, a marketing pitch, or a one-time initiative. It is a long-term investment in trust, relationships, and shared ownership. It demands the humility to listen more than we speak, and the courage to share power with the very communities we seek to serve.

Globally and within India, community engagement is evolving from transactional outreach into an essential piece of core health infrastructure—shifting from top-down directives to co-designed, grassroots solutions that actively integrate the lived experiences of local populations.

The future of healthcare will not be shaped by technology alone, but by the strength of the human connections built around it. The time has come to move beyond talking to communities and begin building with them—because sustainable change is created not for communities, but alongside them.